Ear Examination — OSCE Checklist

Deakin MD2 · BB4 · ENT

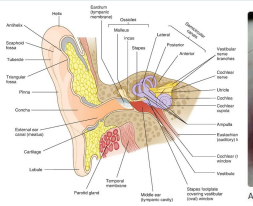
Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Inspect → Palpate → Otoscopy → Whisper test → Weber → Rinne**. *Examine the* **good ear first**.

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain the exam and gain consent; ask about pain
☐	Ask which ear is affected; position seated, ear at your eye level
☐	Equipment: otoscope with range of speculae, 512 Hz tuning fork

2. Examination

	Step	What to do / say	Abnormal
Inspection (external ear)			
☐	Pinna	Size, shape, deformity; trauma (haematoma)	Cauliflower ear
☐	Pre-auricular region	Cartilage tags, sinuses or pits	Syndromes with hearing loss
☐	Skin	Suspicious lesions (SCC, BCC), skin conditions of pinna and canal	Neoplasia, eczema
☐	Scars	Pre- or post-auricular	Previous surgery
☐	Hearing aids	Note and remove before otoscopy	
☐	Discharge	Mucopurulent, serous, blood-stained	Otitis externa / media, perforation



Anatomy of the ear



A weepy otitis externa



An eczematous otitis externa

Otitis externa



Squamous cell carcinoma

Squamous cell carcinoma of the pinna



Preauricular sinus

Palpation			
☐	Tragus and pinna	Gently move — tenderness?	Tender = otitis externa
☐	Mastoid	Palpate behind the ear	Tender = mastoiditis (with otitis media)



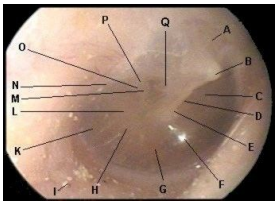
Acute mastoiditis

Acute mastoiditis

Otoscopy			
☐	Speculum	Largest that fits comfortably	
☐	Grip	Hold otoscope like a pen; hand braced against patient's head	Prevents injury if head moves
☐	Straighten canal	Adult: pull pinna up and back; child: pull back only	
☐	Canal	Inflamed? Wax? Discharge? Foreign body?	Otitis externa
☐	Tympanic membrane	Colour, light reflex, short process and handle of malleus, umbo, pars flaccida; move to see whole TM	Bulging, retraction, perforation, grommet



Holding the otoscope — hand braced on head



Right tympanic membrane — landmarks



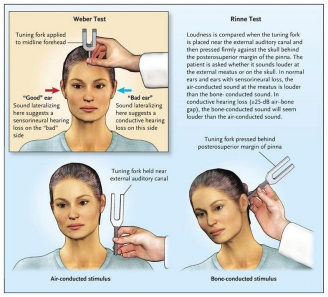
Grommet (ventilation tube)



Common ear drum findings

Hearing			
☐	Whisper test	Mask other ear (rub tragus / fingers); whisper numbers; increase volume if needed; each side	Formal audiology if abnormal
☐	Weber (512 Hz)	Strike fork; base on vertex or midline forehead: "Louder on one side or the middle?"	Lateralises (see table)

☐	Rinne (512 Hz)	Base on mastoid until no longer heard, then prongs beside the ear: "Can you hear it now?"	Positive (AC > BC) = normal



Weber and Rinne tests

3. Closing

☐	Thank patient; clean otoscope speculum; wash hands
☐	Present: "Pinnae normal, no tenderness; canals clear; both TMs intact and pearly grey with a normal light reflex; whisper test normal; Weber central, Rinne positive bilaterally."
☐	Completion: "I would complete the ENT exam (nose, throat, neck nodes) and arrange formal audiometry if indicated."

4. Quick reference

Weber / Rinne (Deakin table)	Weber central	Weber lateralises LEFT	Weber lateralises RIGHT
Rinne AC > BC both ears	Normal	Sensorineural loss RIGHT	Sensorineural loss LEFT
Rinne BC > AC left	—	Conductive loss LEFT	Combined loss
Rinne BC > AC right	—	Combined loss	Conductive loss RIGHT

TM appearance †	Think of
Pearly grey, light reflex	Normal
Red, bulging, lost landmarks	Acute otitis media
Dull, retracted ± fluid level / bubbles	Otitis media with effusion
Hole in TM	Perforation
Small tube in TM	Grommet (not a foreign body)
Swollen canal, debris, tender tragus	Otitis externa

- **Conductive** loss: Weber goes TO the bad ear, Rinne negative (BC > AC).
- **Sensorineural** loss: Weber goes AWAY from the bad ear, Rinne positive (both reduced).
- **False-negative Rinne**: a dead ear hears BC louder via the other cochlea — Weber clarifies.
- Rinne negative needs a conductive loss of ~20 dB or more.